



OFFICIAL

Outpatient Referral Triage and Management Guidelines

1. Purpose

The Outpatient Referral Triage and Management Guideline aims to outline the procedure for triaging and managing all outpatient referrals at Armadale Kalamunda Group (AKG) to ensure:

- prioritisation and allocation of patient referrals to appropriate services within a geographical area and timely manner
- effective and transparent referral management practices which are monitored regularly to ensure capacity meets demand
- A seamless outpatient journey to optimise patient healthcare outcomes.

The document serves to function as an overarching guideline for AKG hospital outpatient clinics. Outpatient referral access criteria are beyond the scope of this document and can be accessed from each Outpatient Specialty clinic or via the [Central Referral Service \(CRS\)](#).

A quick reference guide for clinicians is available at Appendix 1.

2. Applicability

This guideline applies to Medical, Nursing, Allied Health and Clerical Staff working across outpatient services excluding Mental Health at AKG.

3. Outpatient referral management procedure

3.1 Referral Sources

All patients require a written referral prior to receiving an outpatient service at AKG as per the [Outpatient Services Policy](#). Referrals can be received from the following sources:

- General Practitioners (GPs)
- Medical Practitioners /Specialists (from within the hospital, other public hospitals, and private hospitals)
- Health care professionals including nursing staff and allied health.
- Other healthcare providers from the community where appropriate (e.g. optometrists, dental practitioners, midwives, audiologists, & Aged Care Assessment teams.
- Individual self-referrals by the patient or guardian/carer/family member may be accepted in limited circumstances, however, referrals are expected to be initiated by healthcare professionals.

Before referencing this procedure document please ensure that you have the latest version from the [Policy Library](#) information hub page.

3.2 Referral Requirements

Referrals are a form of clinical handover and must contain relevant and sufficient information for safe transfer of care and appropriate clinical triage. Referrals must contain minimum demographic data and clinical information to ascertain clinical acuity and priority.

Referrals must meet the minimum requirements as per [WA public outpatient referral requirements](#) and meet the [Referral Access Criteria](#) (RAC) where established to be accepted into AGK outpatient services.

3.2.1 Minimum Referral Requirements

Referrals must meet the minimum requirements as per [WA public outpatient referral requirements](#) to ensure accurate triage, prioritisation and allocation of the referral. Every effort should be made to increase awareness of referral requirements with referrers so that all patients receive equitable treatment in a timely manner.

To ensure patient receive a timely and equitable service, clinicians may utilise a variety of tools to inform referrers of the referral requirements such as:

- Referral templates with required information
- Regular written & verbal feedback processes
- Clinical support tools such as [Clinician Assist WA](#)
- Returning referrals with feedback or request for more information
- Encouraging use of guidelines relating to investigations relevant to the patient and their initial assessment such as Referral Access Criteria
- Encouraging referrers to contact the outpatient service if the referral urgency needs to be escalated in view of the patient's deterioration.

3.2.2 Referral Access Criteria

Referral Access Criteria (RAC) are mandatory state-wide criteria that apply to all WA public medical led outpatient services. Referral access criteria have been developed for selected medical specialties and outline the clinical criteria (which conditions or presentations are appropriate for referral) and mandatory information such as required investigations & information that must be included in the referral for acceptance by the service. The RAC aims to maximise the value of the patient's first appointment, by ensuring the referral is triaged for the appropriate service and prioritised accordingly.

If the mandatory information is not included in the referral, an explanation must be included in the body of the referral to facilitate acceptance by AGK outpatient services.

Referrals missing information will be rejected and returned to the referrer for re-submission of the referral with requested information. Based on clinical judgement and urgency such as immediate referrals, the referral may be accepted but a request for more information can be made to the referrer.

Additional criteria relevant to AGK clinics should only be applied when the patient referral has been accepted. Should additional information be required, it is the clinician's responsibility for sourcing the information from the referrer or the patient directly.

3.3 Referral Receipt

Referrals can be received in the following ways:

Table 1: Referral Methods

Referral Method	
Central Referral Service (CRS)	Referrals from GPs, private specialists, specialist Medicare Benefits Schedule (MBS) billable clinics and Nurse Practitioners must be sent to CRS. CRS referrals are uploaded to eReferral for ease of access by all clinicians.
eReferral	All internal and inter hospital referrals for WA Department of Health (DoH) hospitals need to be completed electronically on eReferral except for the following: Joondalup Health Campus (JHC) Referrals from private hospitals are to be sent directly to CRS (for medical clinics) or to the service (for non-medical clinics)
Hardcopy referrals (fax/post/email)	Hardcopy referrals can be accepted for non-medical clinics only (nursing and allied health). These are uploaded onto eReferral prior to triaging. These referrals originate from community health service providers and private hospitals which do not have access to the eReferral system.

3.3.1 Central Referral Service (CRS)

The Central Referral Service is a single point of entry for all referrals to WA DoH Medical led Outpatient Services in the Perth Metropolitan area and select regional areas (Collie, Southwest, Great Southern and Pilbara regions). CRS is responsible for processing of all routine medical referrals. Referrals are accepted from GPs, private specialists, specialist MBS billable clinics and Nurse Practitioners.

- CRS utilise the service inclusion criteria/service capability information and postcodes when triaging and allocating referrals. CRS is not responsible for the clinical triage of referrals.
- CRS notify the GP and patient of when a referral has been accepted and which health service it has been forwarded.
- CRS referrals are uploaded to eReferral by the health service providers (HSPs), to provide a single point of entry for clinical triage and ease of access by all clinicians.
- CRS does not accept referrals via email or from patients.
- Referrals sent to CRS should not be older than 3 months at the time of receipt.

Referrals for the following services are out of scope for CRS: mental health services, antenatal & obstetric services, nursing and allied health services, immediate referrals (patient requires review in < 7 days), internal (within WA Health), and select regional specialist outpatient services (Peel Health Campus, Wheatbelt, Goldfields, Midwest and Kimberly regions). CRS will return out of scope referrals to the referrer for re-submission via the correct pathway.

Referrals requiring immediate review (within 7 days) also known as immediate referrals, are to be discussed verbally with the service clinician (and accepted) prior to submitting the referral. The referral needs to be sent directly to the service (not via CRS) to ensure timely patient care.

If the referral is incomplete and not meeting the WA public outpatient referral minimum requirements, then the referrals should be accepted to avoid further delays. RAC should not be applied to immediate referrals as the clinician in receipt of the referral needs to request the relevant clinical information from the referrer.

3.3.2 eReferral

eReferral is an electronic referral system designed to be used by all clinical staff for patients across WA DoH health service inpatient and outpatient services. Clinical staff can view patient referrals across all sites promoting appropriate care and reducing duplication of referrals and services.

AKG uses eReferral to create, access, triage and manage referrals for all outpatient services, and inpatient consultancy services. All requests for consultation and referral to outpatient services must be created or uploaded to eReferral. No hard copy referrals will be accepted outside of this process to ensure standardisation and centralised access.

To ensure equitable access, clinical prioritisation and triaging requirements, internal referrals should not be older than 4 weeks at the time of receipt.

eReferrals should not be printed and do not need to be filed in the patient's digital medical record (DMR) as the electronic record is sufficient. eReferrals can be accessed via WA Health eReferral app, DMR, and iClinical Manager (iCM). All referrals for all sites and case history for the patient can be accessed via eReferral.

The treating clinician needs to access the referral via eReferral and review prior to treating the patient. ALL active and closed referrals for a specialty should be reviewed to ensure information in duplicate referrals (closed referrals) is taken into consideration.

A. Creating an eReferral

Internal referrals are created via eReferral and should only be generated for:

- Patients who require high priority access to services
- Requests for associated care (internal referral requesting services from another outpatient service or specialty relating to the patient's current reason for referral and associated care pathway e.g. assessment, diagnostic tests, investigation, preoperative review).
- Clinical review following an inpatient or Emergency Department admission.

When creating an eReferral for a patient, the "Referral To" site will default to the patient's post code related catchment area for adults. For paediatric patients, the hospital site will default to the paediatric site based on the patient's date of birth (DOB) and postcode. The requestor can change to a different site, if required.

The service delivery mode (e.g. in person, phone call, video call or home visit) can be selected when triaging a patient. The selection will inform clerical of the service delivery mode at the time of booking the appointment.

A “virtual care enabled” flag will appear if a patient has attended a video call appointment in the previous 12 months to assist the clinician in identifying a potential service delivery mode for the patient.

Duplicate eReferral

Patients should only be seen by one service for a specific issue/condition. Requestors should check eReferral if the patient has active referrals (any site) for the same issue, prior to creating a new referral.

Type of Referral

Patient appointment outcome letters or discharge summaries sent to GPs are not regarded as a valid referral for referring to another clinician/service. A separate referral should be completed; highlighting the issues or concerns for referral, need for further investigation, including appropriate patient history and investigations completed.

Associated care referrals are a type of internal referral where support from another specialty is requested in relation to the patient’s existing reason for referral. Support may include:

- Requests for assessment, diagnostic tests or investigations required to support the diagnosis and /or planning the treatment
- Requests for perioperative review or clearance prior to surgery

Associated care referrals are exempt from the “high priority” requirement for eReferrals and are not considered to be a duplicate referral.

If the reason for the internal referral is not related to the patient’s existing reason for referral, then the patient should be directed to their GP to obtain another referral or appropriate care.

Refer to the [eReferrals Login](#) page for access, training, downtime, and site support contacts.

3.4 Triage Referrals

Referrals received via CRS will be uploaded to eReferral by the AKG Referral Officer. Each outpatient service will nominate appropriate staff to triage referrals (gatekeepers). Clinicians are to triage all referrals received in eReferral. Any referrals received via other methods will be uploaded onto eReferral for centralised access and triaging.

A. Triage within the following timeframes:

Table 2: Timeframes for triaging eReferrals

Immediate referrals (care required <7days and require rapid access to services)	< 1 working day after receiving verbal communication from referrer
Urgent referrals	< 1 working day of receipt
Routine / Non urgent referrals	< 5 working days of receipt

- B. Check if the patient has an existing referral for the same issue (your site as well as ALL other sites) to avoid duplication of referrals and service provision. Clinicians can check eReferral case history, DMR and iCM.

If the patient already has an existing referral for the SAME issue (your site or another site) refer to 3.5 Duplicate Referrals.

- C. Referrals should only be accepted for patients that reside in the AKG Catchment area (unless a special EMHS agreement is in place where referrals are diverted). Check postcode and suburb against AKG catchment area using the link below:

[Post Code & Suburb Search](#)

3.4.1 Referral Reassignment

Referrals may need to be reassigned to another service within the same hospital or to another hospital to ensure that the patient is referred to the appropriate service.

If another provider /service is more appropriate for the patient, then the allocated provider should reassign the referral back to CRS (if assigned by CRS) or the referrer (if not processed by CRS).

Should a patient be referred to an outpatient service from Emergency Department or a ward and the patient resides outside the AKG catchment area, the patient may be seen for minimum post care follow up after which their care needs to be transferred to the relevant hospital catchment area, or the patient be discharged.

The clinician is responsible for forwarding/creating a referral to the relevant outpatient service and providing a transfer summary for the patient. The patient needs to be discharged from AKG as per normal discharge procedures for the service.

3.4.2 Referral Acceptance in eReferral

Acceptance of a referral is based on the services inclusion/exclusion criteria, the completeness of the referral and clinical judgement of the triaging clinician.

Service Inclusion/exclusion criteria are reviewed annually and approved by the Manager/Head of Department (HoD). Triaging clinicians can access the criteria via their line manager.

If the referral meets the minimum requirements, RAC (if applicable), and is appropriate for the services, it is to be accepted and triaged by:

- assigning a sub service within the unit (if relevant)
- assigning a responder
- assigning a booking window (priority category) relevant to patient care urgency
- assigning a clinic within the service (if a specific one is required, leave blank if multiple options)
- assigning a service delivery mode
- including important information in the "Notes to Clerk"
 - if an interpreter is required for booking and specify the language required
 - if appointments for multiple clinics are required
 - if the care is related to a compensable claim (Workers Compensation, Motor Vehicle Insurance Trust, Department of Veteran Affairs)

Once the referral is triaged and accepted a booking request is automatically made in WebPAS (Web based Patient Administration System). The clerk will book relevant appointments and notify the patients via letter, SMS and/or telephone. In some services, clinician's book patient appointments and provide clerical staff with appointment details for retrospective data entry.

3.4.3 Assigning priority category (booking window)

To ensure patients are receiving the care required within a clinically appropriate timeframe, gatekeepers (triaging clinicians) are to allocate a priority category in accordance with WA Outpatient Services Policy.

The priority categories (booking windows) available are:

Table 3: Referral Priority Category

Category	Appointment Timeframe
Category 1 Urgent	Within 0-30 Days
Category 2 Semi-urgent	Within 31-90 days
Category 3 non-urgent	Within 91-365 days

Where there is uncertainty about referral priority, the triaging clinician can contact the referrer or patient to discuss the referral and prioritise accordingly. Provision of outpatient services should be based on clinical priority regardless of the referral source (internal vs external referrals).

If a specialist or clinician is unlikely to be able to provide treatment within the recommended timeframe, the referral may be transferred to another specialist or clinician within the same hospital, or another WA DoH hospital if they are able to expedite access to care. Clinician to liaise with Service Managers/Head of Department (HOD) as required.

3.4.4 Active life of referrals

The active life of a referral, regardless of source, is valid for a single course of treatment and for a defined period starting from the date of the first attended appointment with the outpatient service.

Table 4: Active life of referrals

Referral Source	Active life of referral
Internal (within WA Health)	3 months from the date of the first attended appointment
External (outside WA Health)	12 months from the date of the first attended appointment

Referrals longer than 12 months or Indefinite should only be used when the patient requires continuing care and management by an outpatient service.

If a referral's life has expired, the patient is to be notified and will be required to request another referral.

If the patient requires treatment for a different condition than originally referred (except for emergencies and suspected cancers), the patient is required to obtain another referral.

3.4.5 Referral Rejection

Referrals may be rejected if they are:

- inappropriate or do not meet inclusion criteria for the service
- incomplete or poor quality with limited information necessary for triaging
- able to be managed by the referrer with appropriate clinical care advice and guidelines provided when rejecting the referral
- patients who live outside the AKG catchment area
- patient already has an active referral for the same issue within the WA Health system (unless second referral is a transfer of care/shared care from that service
e.g. transition from paediatric to adult care and associated care referral requesting a specialty only available at a different HSP)
- for immediate referrals it is preferable to accept the referral even though it may be incomplete (missing minimum referral requirements or not meeting RAC) and the clinician can request further information.
- for cat 1 /urgent patients it is preferable to:
 - contact the referrer if referral is incomplete rather than rejecting.
 - forward the referral if it has come to the incorrect place (and notify referrer)

Rejected referrals must include comments relevant to:

- reason for rejecting the referral including unavailability of requested outpatient service
- correct service provider/catchment area hospital /specialty for onward referral
- additional information required if rejection is based on lack of information

Referrals that have been rejected in eReferral will appear in the 'rejected' tab and an email will be sent to the referrer notifying them of the rejection.

Clinicians must review the rejected tab regularly in eReferral (for the unit not just their own). Rejected referrals will auto close after 30 days in eReferral if the referral is not actioned, resulting in lack of follow up for the patient. The referrer will lose access to the referral rejection reason and will need to complete a new referral.

Table 5: Referral rejection

Source of Referrals	Medical Clinics	Non-Medical Clinics
eReferral	Reason for rejection & alternative options included in response DoH referrers will be sent an automated notification	Reason for rejection & alternative options included in response DoH referrers will be sent an automated notification
CRS	Referrer notified with rejection reason letter completed by AKG referral office	
External non-WA Health provider	Triaging clinician to inform referrer of the rejection and reason or if forwarding referral, to notify referrer.	Triaging clinician to notify referrer of rejection and reason

3.4.6 Duplicate referrals

Duplicate referrals are referrals that are identical for the same patient, condition, specialty and treatment from the same referring practitioner. Where there are duplicate referrals for a patient, either same service and site, or another service/site: all referrals are to be reviewed to avoid duplication of referrals and service provision. Only one referral per specialty for the same issue can be active at any one time. The following guidelines indicate how to manage duplicate referrals for the same patient:

- **Identical referrals**

The duplicate referral should be triaged as 'no appointment required' with appropriate comments 'duplicate referral.' This will close the duplicate and the initial referral & any appointments will remain active. This response can be viewed by the referrer if the referral originated in eReferral. CRS rejects identical referrals. If external to eReferral (GP or community) the requestor may be notified depending on the service's procedures.

- **Non identical referral with the same urgency and condition**

If the referral is notifying you of additional information and investigations, but does not change the urgency of patient care, the duplicate referral is to be triaged as 'no appointment required' with appropriate comments 'duplicate referral'. The treating clinician can view the additional information. The duplicate referral will be closed but the initial referral & any appointments will remain active. Comments in eReferral can be viewed by the referrer if the referral originated in eReferral. If external to eReferral (GP or community) the requestor will not be notified.

- **New referral with increased urgency**

Duplicate referrals are often received escalating patient care/urgency or providing additional information.

If the patient appointment has not been booked yet, the original referral can be re-triaged, and priority upgraded. The newer duplicate referral is then triaged as 'no appointment required'.

If an appointment has already been booked, the appointment date can be reviewed and changed if required i.e. brought forward as per updated referral, clinician discretion and clinic capacity. Duplicate referral is to be triaged as the updated urgency. Include clerical notes: increased urgency – please review and change appointment date and duplicate referral.

The 'Booked' tab in eReferrals will indicate if an appointment has been booked.

- **New referral with different condition**

Patients may have multiple referrals for the same specialty but for different conditions (e.g. a referral to orthopaedics for a fractured elbow and a total knee replacement). If different clinicians are required to treat the conditions separately, then 2 referrals are appropriate to remain active.

If the 2 conditions can be treated by the same clinician, concurrently, then only 1 referral is to remain active, and patient treated by clinician for both conditions (e.g. a child referred to a paediatrician for two medical issues).

- **Exceptions with duplicate referrals**

Circumstances where duplicate referrals can co-exist with two separate providers include:

- Sub-specialty and/ or expertise (associated care referral), only offered at another hospital site.
- Transition from paediatric to adult care services
- Patient is permanently relocating and transferring between hospital sites (original referral should be closed once patient has been transferred to another hospital site)
- If duplicate referrals exist for two or more outpatient services, the referral relevant to the patient's catchment area should be honoured. The triaging clinician should notify the other hospital, patient and referrer of where the patient will be treated.

It is important to **view ALL referrals** for a patient (active and closed) at each appointment to ensure additional/escalation information is not missed.

3.4.7 Named referrals

Referrals to a named clinician for a patient within AKG catchment area will be accepted by the service. Each specialty is to agree whether named referrals will be allocated to the clinician listed, or any appropriate clinician within the team. As a public WA DoH outpatient service, it is recommended that named referrals are allocated to the team in general, unless based on specific clinician skills set, second opinion and continuity of care.

Out of catchment named referrals should not be accepted unless specific clinician's skill set is required for ongoing clinical care or for continuity of care with the same clinician i.e. clinician see patient previously for recurrent issue.

A named referral also permits a salaried specialist to bill through the Medicare Benefit Schedule (MBS) if they meet requirements as per the [MBS-Billed-Non-Admitted-Services-Manual.pdf](#)

Name referrals intended for the private health sector should be directed to the private provider directly and not via CRS and should not be accepted by WA Health Outpatient Services. Named referrals for private practitioners in WA Health are not prioritised over the waitlist but rather prioritised by clinical urgency.

3.4.8 Multiple Referrals

When a patient requires care from multiple services for the same condition, then a separate referral must be created for each service. If the patient has been referred to a multidisciplinary team or clinic within one specialty e.g. Multidisciplinary Foot Ulcer clinic, then only one referral is required.

4. Financial Classification

4.1 Medicare ineligible patients

AKG is often required to provide medical care for patients who are ineligible for Medicare. Medicare ineligible patients include overseas travellers, international students, foreign defence force and non-permanent residents of Australia who are holders of certain visa classes. Referrer to indicate on referral if patient is not eligible for Medicare benefits.

Clerical staff to liaise with Finance/revenue team on receipt of a referral without a Medicare number to ensure patients are informed of fees and alternative options, if required. Patients must agree to payment for the service, or their referral may be closed, if care is deemed non urgent or able to be obtained elsewhere.

Referrals for urgent treatment (<7days) must be processed and care provided regardless of the patient's financial status as a duty of care.

In services, where clinicians make their own appointments, the Medicare number must be checked (can be seen in demographic section of eReferral or the comments section in iCM) and clerical support staff to be notified if there is no Medicare number entered. Clerical staff to follow up with patient. In all cases, the patient's financial classification should be recorded in WebPAS and DMR.

4.2 Compensable patients

Compensable referrals include patients referred post Motor Vehicle Accidents (MVA), Motor Vehicle Insurance Trust (MVIT), Department of Veteran Affairs (DVA) and Workers Compensation (WC).

Referrals for DVA patients will be assessed through the public referral pathway if the patient chooses to be treated in WA public health service. A funding arrangement between the Department of Veterans' Affairs (DVA) and the State of Western Australia allows for DVA patients to be seen at AKG.

Referrers need to indicate if the patient's condition is related to one of the above in the referral 'details' box. Triaging clinician needs to highlight this in the 'Notes to clerk' box. Clerical staff will then follow up and ensure all appropriate details are obtained and the appropriate financial classification allocated.

5. Management of active referrals

5.1 Appointment bookings

Outpatient appointments will be booked via WebPAS, by clerical staff as per priority category assigned at triage.

Patients are to be seen in order of referral date, within each priority category.

Clinicians are required to inform clerical staff of the outcome of the patient appointment on the day of the outpatient service (e.g.: reappoint with specified timeframe, interpreter required or discharge).

5.2 Appointment cancellation process

5.2.1 Patient-initiated cancellations

When a patient cancels their appointment, AKG has a duty of care to notify the patient of the potential risks to their health because of cancelling or not attending their appointment.

Patients who cancel their appointments repeatedly, due to genuine hardship (memory concerns /medical reasons), misunderstanding of appointments, and other unavoidable circumstances, will have their referral reviewed as per clinician's discretion.

If a patient repeatedly cancels their appointments or cancels twice and defers their appointments, the clinician should consider discharge depending on the patient's validity of reason for cancellations. A patient should be discharged, if the patient has indicated their unavailability for treatment for a period exceeding 90 days unless prior notice and satisfactory reason is provided. The treating clinician should exercise discretion on a case-by case basis to avoid disadvantaging patients. Patient led cancellations and subsequent discharges should be recorded in WebPAS and DMR and the GP / or referrer should be notified.

5.2.2 Clinician initiated cancellations

Clinician's may cancel patient appointments for several reasons such as unplanned leave, personal circumstances etc. The service should notify the patient as soon as possible, advise the patient of the reason for cancellation and plan of action should their condition deteriorate. Their appointment should be rescheduled and recorded in WebPAS.

5.3 Patients who Do Not Attend (DNA) appointments

To ensure an equitable and timely service for all patients, patients who do not attend their clinic appointment without giving the hospital prior notice will be classified as 'did not attend' (DNA).

The hospital has a duty of care to ensure that the patient is informed of the potential risks to their health and their referral being discharged because of cancelling or not attending their appointments.

If a patient DNA's, the clinician is to utilise the allocated appointment time to contact the patient via telephone and undertake a telephone consult if able. This ensures the appointment time is not wasted, appropriate patient care is provided, and an outcome can be decided on the day (e.g.: reappoint or discharge).

To ensure consistent management of active referrals, waitlists and effective service provision, patients will be discharged if they:

- DNA their initial appointment (without a valid reason)
- DNA 2 consecutive follow up appointments (without prior notice)

Patients who DNA repeatedly, due to genuine hardship (memory concerns /medical reasons), misunderstanding of appointments, and other unavoidable circumstances, will have their referral reviewed as per clinician's discretion. If the decision is to discharge patient, the clinician is responsible for documenting in the patient's DMR and notifying the referrer and /or GP.

Discharge letters to be sent to the patient and GP informing them of their discharge and the option of being re-referred, if required.

Patients who are not contactable will be discharged from the outpatient service following the first DNA or second follow up DNA. Every effort will be made to verify the patient's correct contact details via the GP, hospital medical records and Next of Kin, prior to discharging the patient. Evidence of attempts to contact the patient must be shown in DMR as documentation or letters (sent to the patient).

5.4 Patient discharge

Patients should be discharged in the following circumstances:

- Patient no longer wishes to receive treatment at their own choice
- Patient is unavailable for treatment for a period exceeding 90 days
- Patient DNA initial appointment without valid reason and uncontactable
- Patient DNA 2 consecutive follow up appointments without prior notice and uncontactable
- Completion of treatment and patient is safe for discharge
- Two consecutive appointment deferrals
- Patient is receiving treatment for the same condition at another hospital or service can be discharged or removed from the waitlist
- Patient has attended a maximum of two consecutive follow up appointments for a medical clinic. Further follow up appointments require approval from a medical consultant.

Once treatment is completed, patients should be discharged to their primary health care provider or GP in an efficient and timely manner as per [EMHS Discharge Communication Policy](#). This will ensure appropriate use of specialist clinic services, streamlining patient flow and a reduction in waitlists.

The treating clinician should identify patients who are ready for discharge based on the specialty clinic's discharge criteria (if available).

Discharge planning discussions should actively involve the patients and ensure they are informed of the self-management of their condition in conjunction with the patient's GP and /or other relevant health care providers.

The discharge of a patient should be clearly documented in the patient's medical record with a clear plan of onward referrals, if required. A discharge summary or letter should be sent to the patient's GP and/or other health care providers.

5.4.1 SOS Discharge for patients

Patients who do not require a follow up appointment will be discharged (SOS Discharge) from the service with the option of contacting the clinic within 3 months, should their condition change. The referral can be re-activated, and an appointment can be booked.

The SOS discharge process reduces the number of patients attending follow up appointments, unnecessarily. Patients who contact the clinic outside the 3-month timeframe will require a new referral from their GP. Clinicians need to indicate, "Discharge SOS" on their clinic outcome slip or clinic sheet for patients suitable for the discharge SOS process. Any patient that is unsuitable for SOS discharge will be required to obtain a referral from their GP.

6. Outpatient clinic management

Any care provided to a patient who is not a current inpatient, or a current emergency patient is classified as outpatient care.

All outpatient services must be captured in the Web Based Patient Administration System (WebPAS) to ensure adequate tracking of clinical workload and payment for care provided.

Outpatient services and clinic structures are to be reviewed regularly by service providers and the Outpatient Service Manager to ensure appropriateness of naming conventions, model of care and categorisation for funding purposes.

Clinic appointment structures must reflect actual service structure for efficiency of patient bookings and reporting.

Service Managers and HoDs are to review the following on a regular basis in conjunction with the Outpatient Manager:

- Outpatient activity v's budget
- Outpatient Waitlists
- Triaging timeliness
- Timeliness of clinic/discharge letters
- Clinic cancellations
- Service access criteria

7. Roles and responsibilities

Role	Responsibility
Referrers	To assess and refer patients to other services as required. To determine if patients already have active referrals for the same condition Monitor rejected referrals and action accordingly.
Triaging clinician	A clinician within the service or a triage officer who triages referrals and allocates to appropriate services as per service inclusion /exclusion criteria. Monitor referrals and inform HODs of any concerns. Develop and review triaging procedures
Clinicians	Complete patient care in a timely manner, ensuring appropriate communication with referrers and GPs. Manage active patient referrals as required in response to DNA/Cancellations.
Clerical Staff	Book appointments for patients as per triaging requests. Book interpreters as required. Clarify the financial classification non-Medicare and compensable patients as required.

Role	Responsibility
	Monitor patient appointments and referrals and inform HOD or triaging clinician if any concerns.
HoDs	Manage any concerns from clinical staff, clerical staff and triaging officers. Monitoring of waitlists, service provision & timeliness of discharge communications. Regular review of service criteria and clinic structure/models.
Outpatient Manager	Manage any issues raised by clinical or clerical staff. Monitor and report on activity, budgets, waitlists & patient feedback.

8. Legislative context

The following documents are required to give effect to this policy:

- HDWA
 - [Specialist Outpatient Services Access Policy OD 0430-14](#)
 - [Central Referral Allocation – Outpatient Services OD 0503/14](#)
 - [Outpatient Services Policy](#)
- EMHS
 - [Discharge Communication \(Written\) Policy EMHS:12](#)

9. Supporting Information

The following documents support the implementation of this policy:

- HDWA
 - [eReferrals Resources](#)
 - [WA public outpatient referral requirements](#)
 - [Referral Access Criteria](#)
 - [Outpatient Services Guideline](#)
- AKG
 - [Outpatient Telehealth Policy](#)
- Other
 - [MBS Billed Non-Admitted Services Manual](#)
 - [Catchment Post Code Map and Post Code Suburb Search](#)
 - [Communicating for Safety Standard](#)

10. Compliance, monitoring, and evaluation

Compliance of this policy will be monitored by the Outpatient Manager in conjunction with the Head of Department/Manager.

11. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 5 – Action 5.5 Collaboration and teamwork

- The health service organisation has processes to:
 - Support multidisciplinary collaboration and teamwork
 - Define the roles and responsibilities of each clinician working in a team
- Standard 6 – Action 6.3 Partnering with consumers
- Clinicians use organisational processes from the Partnering with Consumers Standard to effectively communicate with patients, carers, and families during high-risk situations to:
 - Actively involve patients in their own care
 - Meet the patient's information needs
 - Share decision-making
- Standard 6 – Action 6.4 Organisational processes to support effective communication
- The health service organisation has clinical communications processes to support effective communication when:
 - Identification and procedure matching should occur
 - All or part of a patient's care is transferred within the organisation, between multidisciplinary teams, between clinicians or between organisations, and on discharge
 - Critical information about a patient's care, including information on risks, emerges or changes

12. Document contact

Director Operations & Corporate Operations

Enquiries relating to this guideline may be directed to:

Title: Outpatient Services Manager
 Department: Corporate Operations
 Email: AHS.OutpatientClinicManagement@health.wa.gov.au

13. Review

This guideline will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V4	22 01 2026	22 01 2029	Full early review, updates in processes.

14. Authorisation

This guideline has been authorised and issued by:

Authorised by	Leanne Watson – A/ Director Mental Health and Ambulatory Care
Authorisation date	19 01 2026
Published date	22 01 2026

Appendix 1: Quick reference guide for clinicians

